P.I.

80 mm

OTSU-D5,NS 5% DEXTROSE AND 0.9% SODIUM CHLORIDE INJECTION U.S.P.

COMPOSITION:
Each 500mL solution contains 25.00 g Dextrose Monohydrate and 4.50 g Sodium Chloride.

DESCRIPTION:
Clear, colorless, sterile and pyrogen free solution.

DRUG ACTION:
Dextrose and Sodium Chloride injection have value as a source of water, electrolytes and calories. They are capable of inducing diuresis depending of the clinical conditions of patient.

INDICATION:
As an intravenous supply of nutrition, restabillize the electrolyte balance for dehydration.

DOSAGE:
Intravenous administration, the dosage depending on the patients condition individually.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE (S):
- Infusion of dextrose much above 800 mg per kg body weight per hour, dextrose will appear in the urine, causing hyperglycemia.
- Large doses will cause accumulation and edema, hypokalemia.Antidote: Insulin injection.

WARNING
- Dextrose and Sodium Chloride Injection should be used with great care. If at all, in patients with congestive heart failure, severe renal insufficiency, and in clinical states in which there exists oedema with sodium retention.
- Dextrose Injection with low electrolyte concentrations should not be administered simultaneously with blood through the same administration set because of the possibility of pseudo agglutination or hemolysis. The container label for those injections bears the statement: Do not administer simultaneously with blood.
- The intravenous administration of Dextrose and Sodium Chloride Injection can cause fluid and/or solute over loading resulting in dilution of serum electrolyte concentration, over hydration, congested states, or pulmonary oedema. The risk of dilutional states is inversely proportional to the electrolyte concentration of the injection. The risks of solute overload causing congested states with peripheral and pulmonary oedema. It's directly proportional to the electrolyte concentrations of the injections.
- Excessive administration of Dextrose and Sodium Chloride Injection may result insignificant hypokalemia.
- In patients with diminished renal function, administration of Dextrose and Sodium Chloride Injection may result in sodium retention.
- Parenteral drug products should be inspected visually for particulate matter and discoloration prior to administration whenever solution and container permit.
- Additives maybe incompatible. Complete information is not available. Those additives known to be incompatible should not be used. Consult with pharmacist, if available. If, in the informed judgment of the physician, it is deemed advisable to introduce additives, use aseptic technique. Mix thoroughly when additives have been introduced. Do not store solutions containing additives.

PRECAUTION
- Clinical evaluation and periodic laboratory determinations are necessary to monitor changes in fluid balance, electrolyte concentrations, and add base balance during prolonged parenteral therapy or whenever the condition of the patient warrants such evaluation.
- Cautions must be exercised in the administration of Dextrose and Sodium Chloride Injection to patient's receiving corticosteroids or corticotrophin.
- Dextrose and Sodium Chloride Injection should be used in caution in patients with overt or subclinical diabetes mellitus.
- Pregnancy: Teratogenic Effects

  Pregnancy category C. Animal reproduction studies have not been conducted with Dextrose and Sodium Chloride Injection. It is also not known whether Dextrose and Sodium Chloride Injection can cause fetal harm when administered to a pregnant woman or can affect reproduction capacity.
- Dextrose and Sodium Chloride Injection should be given to a pregnant woman only if clearly needed.
- Do not administer unless solution is clear and seal intact.

SIDE EFFECT
- No significant side effect appears, but on long duration of administration blood picture should be monitored closely, so that no imbalance of the blood electrolyte solution occur.
- Large doses will cause accumulation and oedema.
- Low pH may cause thrombophlebitis.
- Reactions which may occur because of the solution or the technique of administration include febrile response, infection at the site of injection, venous thrombosis or phlebitis extending from the site of injection, extravasation and hypervolemia.
- If an adverse reaction does occur, discontinue the Infusion, evaluate the patient, institute therapeutic countermeasures and save the remainder of the fluid for examination if deemed necessary.

CONTRAINDICATION:
- Hypertensions, acidosis, hypokalemia, diabetes, patients with glucosegalactose malabsorption syndrome.

DRUG INTERACTION:
- If use Natrium Tiopenton as infusion, observe on the possibilities of precipitate.
- Preferable do not add to blood for infusion, because blood cell flocculation and the possibilities of hemolysis could occur.

PACKAGE:
Plastic bottle of 500mL.

STORAGE:
Store below 30°C.

Keep medicine out of reach of children
Jauhi daripada Kanak-kanak

Do not use if bottle is leaking, solution is cloudy or contains foreign matter.

"Controlled Medicine/Ubat Terkawal"

ON MEDICAL PRESCRIPTION ONLY

Otsuka

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CILBSXXXXX

210 mm